

Who Pays After Medicaid Expansion?

Coverage, spending and utilisation across payers in US states, 2010–2018

Emily Johnson

Wennberg International Collaborative · September 2026

The question

In 2014, the Affordable Care Act offered states the option of expanding Medicaid coverage and opened the Marketplace for private insurance. As a result, total Medicaid spending in expansion states increased.

We investigate what happened to enrollment, spending and utilization during Medicaid expansion for Medicaid, Medicare, and private insurance across states.

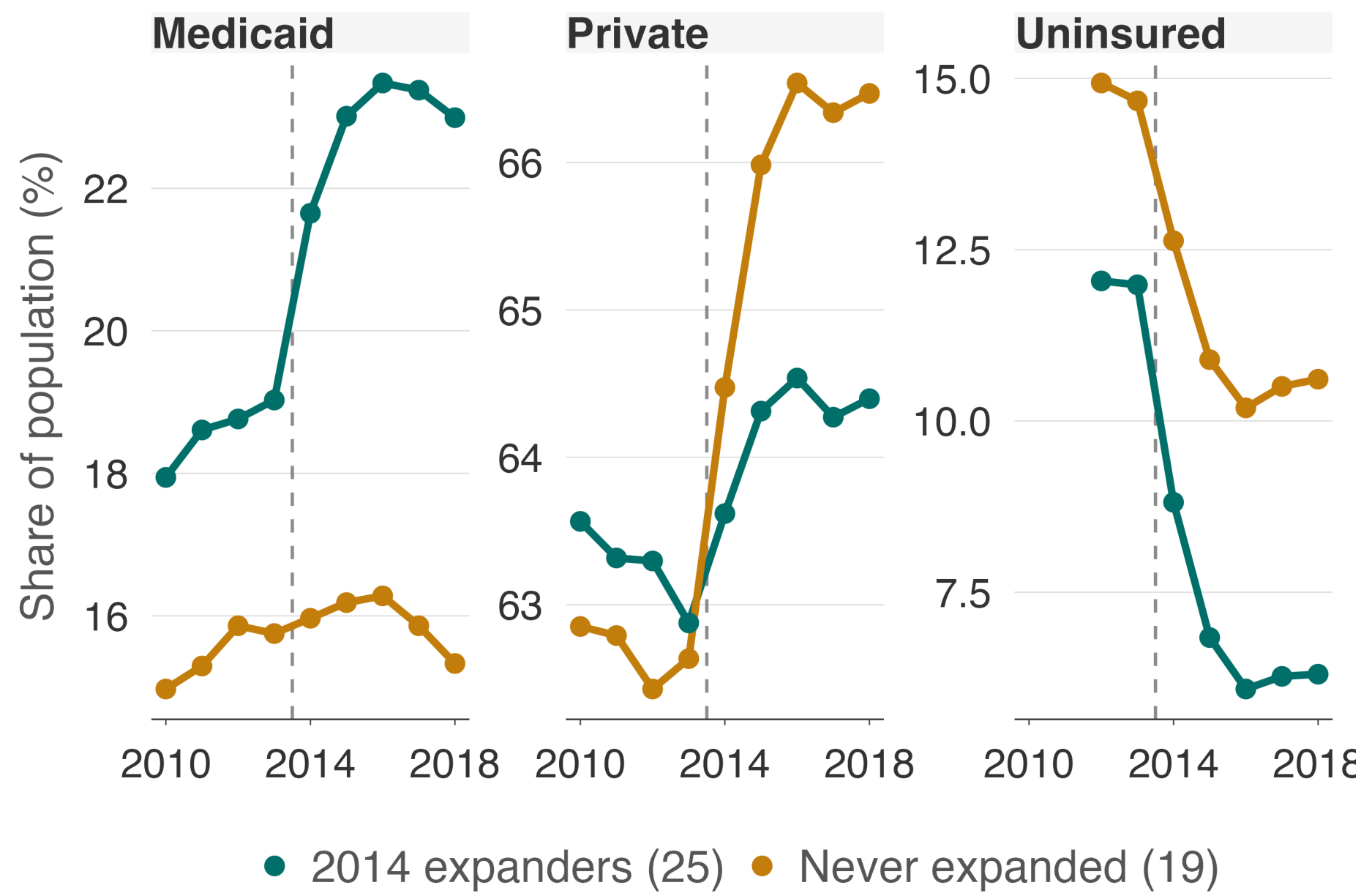
Data and design

IHME Disease Expenditure Project (DEX): state-level spending and encounters by payer and setting, 2010–2018. Uninsurance from CPS.

Difference-in-differences comparing the **25 states that expanded on 1 January 2014** with the **19 that never expanded**. Callaway and Sant’Anna (2021) group-time estimator, never-treated controls, adjusting for poverty, median income, pre-expansion uninsurance and age structure. Standard errors clustered by state, multiplier bootstrap.

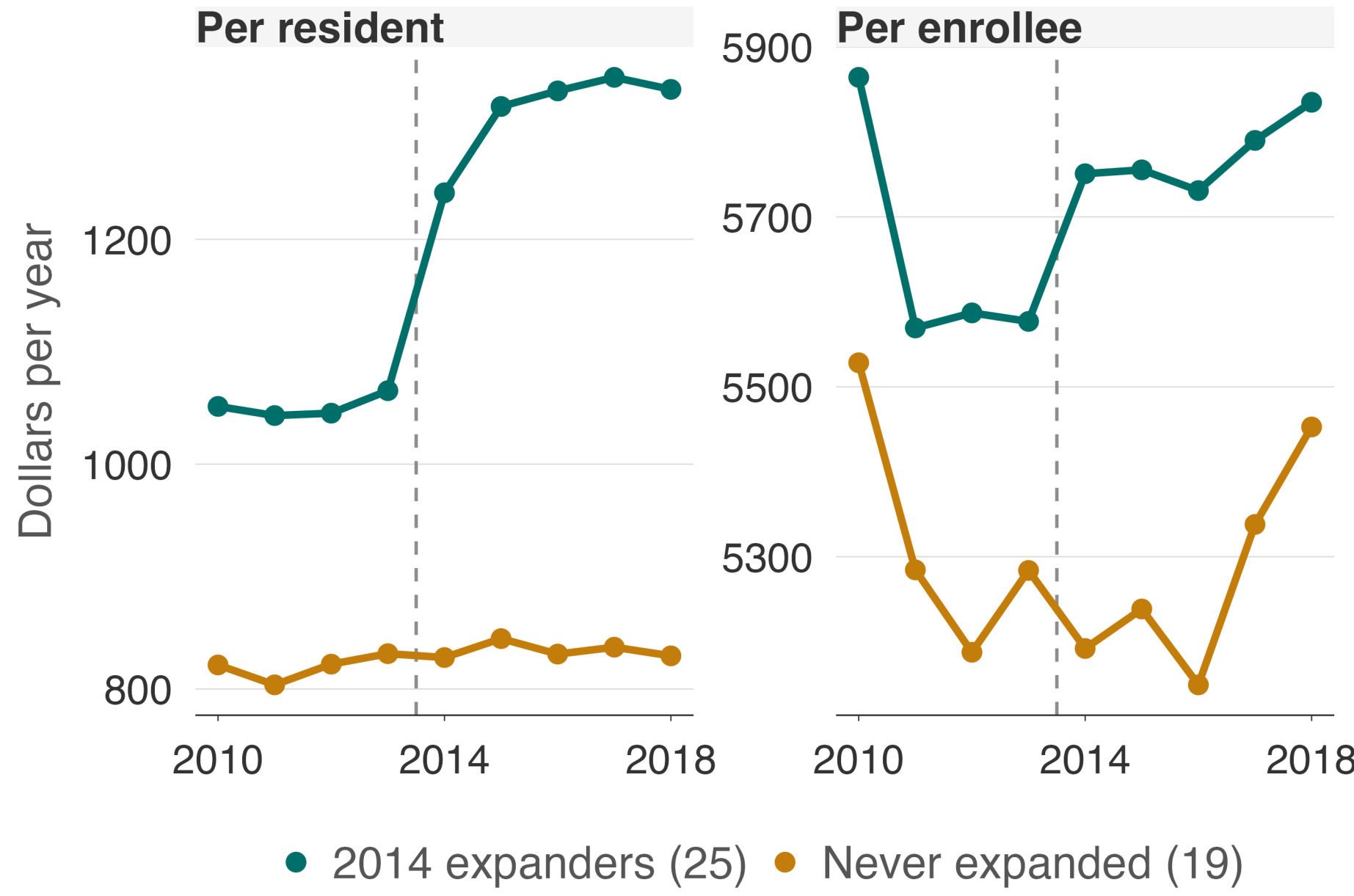
Later-expanding cohorts are excluded due to small sample size ($n \leq 3$ each year).

1. Expansion covered the uninsured



Medicaid rose **3.7 points**. The uninsured rate fell **1.4 points** further than in control states. This is the first stage, and it is the part nobody disputes. The rest of the poster asks what the money did.

2. The new enrollees were not free riders

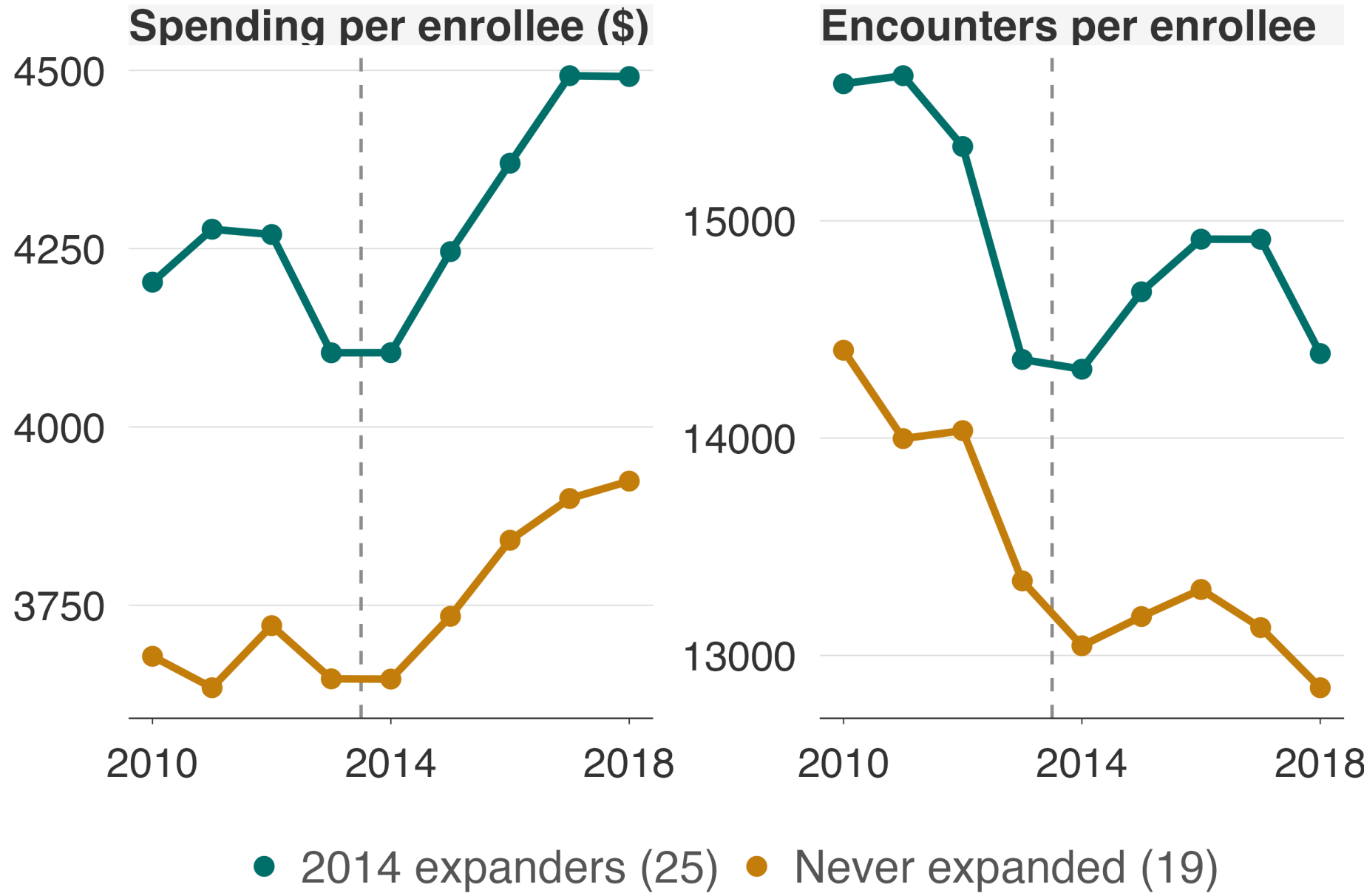


Enrollment rose **19.9%** and spending per resident rose **18.0%** (SE 3.9). Spending per enrollee did not follow: **-3.3%** (SE 4.4).

Had the people expansion enrolled cost nothing to cover, spending per enrollee would have fallen **16.6%**. It did not. That is rejected at **p = 0.002**. Equal cost with the existing caseload is *not* rejected ($p = 0.46$).

Against a pre-expansion **\$5,649** per enrollee, the marginal enrollee cost roughly **\$4,500** — about 80% of an incumbent, and not distinguishable from 100%.

3. Private insurance did not shrink to pay for it



Private coverage rose in both groups — 62.9 to 64.4% in expanders, 62.6 to 66.5% in controls. The **-2.2 point** estimate is control states gaining faster, not expansion states losing. Marketplace subsidies cover 100–138% FPL where Medicaid does not.

Private spending per enrollee **+4.8%** (SE 2.8) and encounters per enrollee **+3.3%** (SE 2.4). Neither reaches significance alone.

Because both margins move together, this is **use, not price**. The most likely reading is dilution of the *control* group’s risk pool as lower-utilising marketplace enrollees joined it.

4. Nothing else moved

Medicare spending per resident **+0.4%** (SE 1.3) — nothing, on any margin. Private spending per resident **+1.7%** (SE 2.4).

Of the **36 spending estimates** below, three survive correction for multiple comparisons, and all three are Medicaid spending per resident. Among the **24 non-Medicaid** estimates, one clears 5% before correction and **none survives it**.

Spending per resident

Setting	Medicaid	Medicare	Private
All care	+18.0 (3.9) *	+0.4 (1.3)	+1.7 (2.4)
Ambulatory	+23.6 (5.8) *	-0.9 (1.2)	+2.1 (2.4)
Emergency	+21.7 (8.1)	+2.9 (4.9)	-0.2 (1.9)
Inpatient	+22.5 (4.5) *	+0.5 (1.4)	+1.2 (3.4)
Nursing	+2.0 (6.9)	+3.0 (5.6)	+4.6 (7.2)
Pharmacy	+11.1 (12.8)	+0.8 (2.9)	+0.8 (5.2)

Spending per enrollee

Setting	Medicaid	Medicare	Private
All care	-3.3 (4.4)	+1.5 (0.8)	+4.8 (2.8)
Ambulatory	+0.0 (6.2)	+0.9 (0.8)	+5.3 (3.0)
Emergency	-2.0 (7.8)	+3.8 (2.7)	+3.2 (2.2)
Inpatient	+4.4 (4.9)	+0.8 (0.9)	+4.0 (3.6)
Nursing	-14.4 (9.7)	+4.1 (4.9)	+7.3 (7.3)
Pharmacy	-22.5 (18.0)	+3.2 (1.4)	+4.2 (4.7)

Effect as % of the pre-2014 level (SE). * survives Benjamini–Hochberg correction across the 36 estimates. Every Medicaid model passes the parallel-trends pre-test; private is borderline at $p = 0.07$.

Limitations

Estimates describe 2014 expanders and non-expanders only.

Out-of-pocket spending is not included in analysis. Uninsured spending and out-of-pocket deductibles or copays for the insured cannot be distinguished in DEX.

Private coverage changes cannot be attributed to individual switching.

Expansion’s new enrollees cost about as much as the old ones, and private insurance did not shrink to pay for them.

Medicaid spending per resident rose 18%; per enrollee it did not move. Private coverage grew in expansion states too — just more slowly than where the marketplace did the covering.